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Plain-language summary for patient and caregiver

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What this page is

This is the patient and caregiver version of a longer, more technical report Libby wrote about treatment options for your cancer. Libby is a research tool, not a doctor. It read the records you shared, went through the published studies and open clinical trials for treatments aimed at your cancer's specific features, and ranked what it found. This page walks through that list in plain language: what each option is, what the studies actually showed, the main risks, and where the evidence is thin. It covers the experimental and feature-targeted side of the picture; the standard approved treatments for this disease are tracked separately and belong to the conversation you have with your own oncology team.

One thing before anything else: nothing on this page asks you to wait. The tests described below run alongside your first chemotherapy cycle, not in front of it.

What we know about your cancer

You already know the hardest part of this story, so I will say it once, plainly, rather than treat it as a footnote. The tumour removed from your uterus in 2024 was read as benign. It was not, and the error was found only this summer, when the spots in your lungs were biopsied and the original tissue was re-examined. It cost you 23 months. Nothing about that was your fault, and several of the practical steps below exist specifically to put that 2024 surgery's records and stored tissue to work for you now.

The diagnosis is high-grade uterine leiomyosarcoma, a rare cancer arising from the muscle wall of the uterus, and it has spread to both lungs. On scans the lung disease has an unusual, diffuse appearance, which matters later when we talk about measuring response and about ever treating the lungs directly. You have never smoked. You live at roughly 8,500 feet, which shifts what "normal" looks like on blood counts and oxygen readings, so every lab result in this treatment should be read against altitude-adjusted ranges rather than sea-level ones.

Two more facts shape the list. First, you have had no chemotherapy or any other body-wide treatment yet, so every first-line option is still fully open. And for what small comfort it offers, the fact that the lung disease took roughly two years to declare itself, rather than weeks, says something about its pace. Second, almost nothing about the tumour's molecular makeup has been measured. The one result that exists is hormone receptors: the tumour tested negative for both estrogen and progesterone receptors (under 1% of cells stained), which closes the door on hormone-blocking treatment, an approach that only helps when those receptors are strongly present. One caveat worth chasing: that stain seems to have been run on the 2024 tissue, and a stain that failed technically looks identical to a true negative. It is worth confirming the test's controls worked, and if there is any doubt, repeating it on the 2026 lung biopsy. Everything else, including the DNA-repair status that decides one of the options below, is unmeasured. Measuring it is itself the first step.

What you told us matters most

Here I have to be honest about a gap: nobody has asked you. The preferences behind this report are entirely assumptions, made at intake so the analysis could run. They assume you would weight effectiveness heavily against side effects, that you have no hard lines on toxicity, and that you would consider clinical trials. None of that came from you. A 39-year-old preparing for aggressive chemotherapy plausibly wants maximum control of this disease, but that is an inference, and it should not harden into your chart without you saying it.

Four questions in particular are yours to answer, and they come back at the bottom of this page: whether the trade the planned chemotherapy offers, its real benefit against its real toxicity, is one you accept; whether you want clinical trials at all, and how far from home you would travel for one; what the hormonal aftermath of treatment means to you at 39, since a hysterectomy does not settle that question and no one has raised it; and how much insurance fighting you are willing to absorb, because your first treatment already involves some.

The first step: tests that run alongside treatment, not before it

The top item on Libby's list is not a drug. It is a package of tests, and putting it first does not mean delay. Nearly all of it runs in parallel with your first chemotherapy cycle, none of it is toxic (blood draws, a heart ultrasound, a CT scan, stains on tissue already sitting in a pathology lab, and two records requests), and most results come back within one to three weeks while treatment is already under way.

A few items genuinely cannot be done later, and these are the only ones that touch the treatment calendar:

- A heart ultrasound (echocardiogram) before the first dose of doxorubicin. Doxorubicin can strain the heart, the trial your regimen comes from required this scan before starting, and once the first dose is given the clean "before" picture is gone forever.
- A CT scan that measures the lung tumours before treatment starts, so that every later scan has something honest to compare against.
- A blood test for an inherited change in a gene called TP53, which guidelines say should happen before this kind of chemotherapy. This one is a decision, not a formality. If it finds an inherited change, that information touches your relatives, who may want testing of their own, and it could reshape how much doxorubicin your team is willing to give you over a lifetime. Whether you want that knowledge at all is your call, and the consent conversation should come before the needle.

The rest runs in the background while chemotherapy proceeds:

- A single testing package on the stored 2024 tumour tissue: DNA and RNA sequencing plus a set of stains. This is what opens or closes almost everything below. It answers the DNA-repair question that gates Option 2, checks the proteins behind Options 3 and 5, and looks for a few rare any-cancer-type markers that, if positive, would open approved treatments through your regular care team. One practical note your team should handle in a single conversation: the 2024 tissue block is finite, so all the orders should be planned together before any one test uses it up.
- The operative report from your 2024 surgery. The load-bearing question is whether the tumour was removed whole or broken up inside the abdomen (called morcellation). If it was broken up, the chance that cells seeded the abdomen is meaningfully higher (about 44 in 100 versus about 13 in 100 in one comparison), and your team would want to check the abdomen carefully before ever planning treatment aimed at the lungs.
- Baseline blood counts, lung function, and oxygen readings, all recorded with your altitude noted, so that later changes on chemotherapy are read against your real baseline.
- The hormone-receptor provenance check described above.

The options

These are in Libby's rank order. The ranking reflects the strength of the evidence, the risks, and the practical path to each option. It is not a verdict, and your oncologist may reasonably weigh things differently.

Option 1 — Doxorubicin plus trabectedin, then trabectedin maintenance

This is the treatment your sarcoma oncologist has already planned, and it is also the best-evidenced treatment in this disease. Those two things do not always coincide. Here they do.

In the one large randomized trial, 150 people with newly diagnosed metastatic leiomyosarcoma received either doxorubicin alone, the long-time standard, or doxorubicin plus trabectedin followed by trabectedin maintenance. On the combination, half the patients went about 12 months before the cancer grew, versus about 6 months on doxorubicin alone. Longer term, half the people on the combination were still alive at about 33 months, versus about 24 months on the single drug. About two-thirds of the trial's patients had uterine primaries like yours; the trial was not sized to give that group its own precise number, but nothing suggests the benefit skipped them.

The price is heavy, and you should hear it in full. About 8 in 10 people on the combination had a severe drop in infection-fighting white cells, nearly half had a severe drop in platelets, and about 3 in 10 developed a fever while their white cells were low, which means hospital admission. That number has direct consequences for how cycle 1 gets written. Preventive white-cell booster injections should be in the orders for the very first cycle, not added after trouble arrives. And you need a written when-to-call plan for fevers before you leave the clinic. Doxorubicin can also injure the heart in a way that adds up over a lifetime of exposure, which matters more at 39 than at 70, so the running dose plan and a protective drug called dexrazoxane belong in the conversation before cycle 1. One altitude-specific trap: because your blood counts run naturally high at 8,500 feet, chemotherapy anemia can hide inside a number that looks normal by sea-level standards. The altitude-adjusted baselines from the workup are what make it visible.

Two more things to know. First, insurance. Trabectedin's official approval is for use after another chemotherapy has been tried, and you have not had one, so this first-line use is "off-label." That is a paperwork status, not a scientific one; the trial behind the regimen was itself first-line. But it makes an initial denial predictable, so the request should be filed as off-label from day one with the trial publications attached. A denial discovered late, at the pharmacy stage, is the one thing on this page that could actually delay your treatment. Second, the maintenance phase, likely the part you will be on longest, is the least-tested part of the plan. No study has compared continuing trabectedin against stopping once the cancer is controlled, and the maintenance side-effect details were never fully published, so monitoring borrows from trabectedin's record as a single drug: liver enzymes and a muscle enzyme called CK on a regular schedule. That gap is a reason to revisit, every few cycles, whether staying on maintenance is still earning its keep. It does not undermine the decision to start.

Whether you accept this regimen's trade of benefit against toxicity has not been asked. It should be, before the first bag hangs.

Option 2 — Olaparib plus temozolomide, queued for the day the first regimen stops working

This option is only available if the DNA-repair test comes back positive. A minority of leiomyosarcomas lose part of their machinery for repairing damaged DNA; doctors call this homologous recombination deficiency, or HRD, and the BRCA genes are the best-known members of that machinery. The tumour testing in the first step looks for exactly this, which is why it goes out now: so the answer is already in your chart on the day it matters. If the test comes back showing normal repair, this option comes off the table. The other options on this page do not depend on it and still stand, and standard next-line treatments for this disease exist as well; those sit outside what this page ranks and belong to a separate conversation with your oncologist.

What the evidence shows: in a small study, 22 women with previously treated uterine leiomyosarcoma took olaparib, a pill that blocks a backup DNA-repair route (a PARP inhibitor), together with temozolomide, an oral chemotherapy. Tumours shrank in about 1 in 4 of them, and half the women went about 7 months before the cancer grew, a good result in a setting where single drugs often manage far less. A deeper analysis hinted that women whose tumours carried the repair defect did clearly better than those whose tumours did not, roughly 11 months versus 5 before growth. That hint came from 18 evaluable patients and used a specialized research assay that no clinical laboratory currently offers, so the panel your team can actually order is a reasonable stand-in rather than the same test. A positive result improves the odds without promising the 11-month figure. And keep the size of the whole thing in view: 22 women, everyone got the drug, no comparison group.

The costs. This combination is hard on bone marrow, and yours would just have absorbed the induction above, so documented count recovery comes before any prescription. The study reported its severe white-cell drops two different ways in its own publications, 75% in one place and 59% in another; that is a reporting problem worth knowing about, and the honest summary is "a majority." About a third of patients had severe platelet or red-cell drops. Finally, the dedicated trials are closed to new patients, so getting this combination today means another off-label insurance case, one that only holds together if a positive repair-test result is sitting in your chart. Which is, again, why the test goes out now.

Option 3 — B7-H3 antibody-drug conjugate (a trial door, opened or closed by one stain)

This option carries caveats. Here is the tradeoff to weigh.

An antibody-drug conjugate is a targeted way of delivering chemotherapy: an antibody finds a particular protein on the cancer cell's surface and drags a potent drug payload to it. The protein here is called B7-H3, and the case for it in your disease is that it is very likely present. In a survey of 153 soft-tissue sarcomas, 97 in 100 carried B7-H3, and about 69 in 100 carried it at high levels. A single stain on your stored 2024 tissue, costing about two slides, answers whether yours does.

The caveats are substantial. A protein being present is not the same as the cancer depending on it, and related research suggests these drugs do little when the target sits at low levels, so a strongly positive stain is what would matter, not a barely positive one. No B7-H3 drug has any reported results in leiomyosarcoma, so there is no response rate to quote you, only the logic of the target. The open study is an early dose-finding trial: everyone gets the drug, there is no comparison group, and early participants can receive doses later shown to be too low. And an earlier drug in this same class was halted after safety findings in a prostate-cancer trial; whether that problem belonged to that one drug or travels with the class is not yet answered.

What Libby actually recommends here is modest. Run the stain now, while it is nearly free. Book nothing until your blood counts have recovered from induction and until someone has asked you directly whether you want a trial and how far you would travel for one. If the stain comes back strongly positive, this is probably the most reachable experimental door for a second or third line.

Option 4 — FAP-targeted radioligand (a trial door, opened or closed by one scan)

This option carries caveats. Here is the tradeoff to weigh.

A radioligand is a small radioactive drug that circulates until it finds its target protein, then irradiates the cells immediately around it. The target here is FAP, a protein sarcomas are unusually rich in; in this disease the cancer cells themselves often carry it, at meaningful levels in roughly half of sarcomas. What makes this branch easy to

explore is that the eligibility question is answered by a special PET scan rather than by tissue. The scan either lights up, meaning the target is there, or it does not, and a negative closes this door cleanly at no cost to you.

The concern is specific and worth taking seriously. FAP also sits on support cells inside bone marrow. In mouse experiments, an earlier FAP-directed treatment, an engineered cell therapy and so a different technology, caused severe marrow damage. A short-lived radioactive drug that clears through the kidneys is a genuinely different exposure than an engineered cell that persists in the body, but no human data yet says how different, and the trial that would generate that data has not reported. Add the obvious: your marrow will just have taken the heaviest chemotherapy in this disease, and no baseline blood count even exists yet. There is also no human efficacy data for this drug in sarcoma; it too is an early dose-finding study.

The recommendation is again screen-first. If and when the first regimen stops working, get the PET scan; it can piggyback on the restaging scan you would have anyway. Any actual treatment decision waits on the trial's own safety data and on your recovered blood counts.

Option 5 — REC-617, a CDK7 inhibitor, after RB1 testing

This option carries caveats. Here is the tradeoff to weigh.

REC-617 blocks a protein called CDK7 that fast-dividing cancer cells lean on. The trial taking leiomyosarcoma patients requires a tumour stain showing loss of a gene called RB1, and there is an oddity in how that gate works in your disease: RB1 loss is nearly universal in leiomyosarcoma. So the stain almost always comes back "eligible," and what it routes you into is an early-phase trial arm with no leiomyosarcoma results at all. The scientific idea behind the drug was worked out with a different compound in a different cancer, small-cell lung cancer, and nothing yet shows it transfers. Then the practical load: it is a dose-finding study at one single hospital, a long trip from a home at 8,500 feet, and it requires a fresh biopsy and precisely measurable disease, both uncertain given the diffuse way your lung disease looks on scans.

What survives all that is cheap and worth doing: the RB1 stain joins the bundled tissue work at essentially no cost, and it keeps this door real rather than theoretical for a second line, if, and only if, you decide trial travel is something you want.

Option 6 — Abemaciclib plus gemcitabine, in a first-line trial

This option was considered but not recommended. It is on the page because Libby surfaces what was weighed and rejected, not just what was kept. Here is what the concern was.

This trial pairs gemcitabine, a chemotherapy with a track record in this disease, with abemaciclib, a pill that pauses the cell-division cycle. It was the only recruiting first-line trial open to leiomyosarcoma patients who have never had chemotherapy, which is the only reason it was ever on the list. The problem is the schedule. Laboratory work suggests that a cell-cycle-pausing drug given at the same time as chemotherapy can actually shield some cancer cells from that chemotherapy. The version of this idea that worked in leiomyosarcoma lab models gave the drugs in sequence, one after the other, and this trial gives them together. On top of that, joining a first-line trial would spend something you only have once: your never-treated status is exactly what qualifies you for the proven regimen at Option 1, with its roughly 33-month survival result. Trading that for an untested combination whose own schedule argues against its rationale is a poor exchange.

One thing from this option deserves to outlive it: it existed partly to force the question of whether you want trials at

all. That question still has not been asked, and it should be, whatever happens to this particular study.

Treating the lung spots directly: a maybe-later, not a plan

You may wonder why nothing above involves operating on or irradiating the lung tumours themselves, since your consulting oncologist has explicitly kept that on the table. The answer is that two gates sit in front of it, and neither has been passed. First, chemotherapy has to work. The surgical and radiation studies that report good local results were done in people with a small number of well-defined lung nodules, and your current pattern, spread through both lungs with a diffuse appearance, is what those studies screened out. If induction shrinks and sharpens the picture, the question gets asked again for real. Second, the 2024 operative report has to answer the morcellation question, because if tumour was broken up during that surgery, the abdomen needs a careful look before anyone plans lung-directed treatment. So: kept open, and meant seriously, but a question for later rather than a plan for now.

Other ideas Libby logged and set aside

Behind the six options above sit sixteen more possibilities the search turned up, kept on the record so nothing disappears silently. None is a live door today. Two are alternate versions of Options 3 and 4 run by other sponsors; one is the discontinued predecessor drug described under Option 3; several are early-stage ideas with no results in this disease to weigh; and the rest have no way in, because the trial is closed, restricted to a different sarcoma type, running only overseas, or built for an anatomy you do not currently have. Two are worth watching from a distance rather than joining: a trial abroad asking the exact continue-or-stop maintenance question Option 1 leaves open, and a closed trial of a similar first-line combination whose eventual results will either back up Option 1's evidence or complicate it.

Questions to ask your oncologist

1. Is my echocardiogram scheduled before the first doxorubicin dose, and are the baseline CT and blood work ordered so that none of these tests delays cycle 1?
2. Will preventive white-cell booster injections be in my first cycle's orders, and exactly when, and whom, do I call if I get a fever at home?
3. Has the insurance authorization for this regimen been filed as off-label with the trial publications attached, and what is plan B if it is denied on the first pass?
4. Before the germline TP53 blood draw: what would a positive result mean for me, for my doxorubicin planning, and for my family, and can I decide after that conversation whether I want the test at all?
5. Has the DNA-repair (HRD) panel been sent from my 2024 tissue, and when will it be back? If it shows normal repair, what does my second-line plan look like instead?
6. Was my hormone-receptor stain run on the 2024 tissue or the 2026 lung biopsy, did its controls work, and should it be repeated on the newer sample?
7. Has the operative report from my 2024 surgery been requested, and was the tumour morcellated? If it was, what changes about my scans and about ever treating the lungs directly?
8. Given that my blood counts and oxygen readings run differently at 8,500 feet, will my labs be read against altitude-adjusted ranges, and how will we decide, cycle by cycle, whether staying on trabectedin maintenance is still worth it?

And four questions to expect, or to raise yourself, because the answers are yours alone: whether Option 1's trade of benefit against toxicity is acceptable to you; whether you want clinical trials, and how far you would travel for one; what the hormonal aftermath of treatment means to you at 39; and how much insurance fighting you are willing to take on.

Sources

PMIDs cited by the recommendations: 12907617, 22751436, 23712432, 25593184, 27602331, 28454547, 28619757, 31883968, 32457520, 33344901, 35608703, 35835135, 36624315, 37143137, 37333052, 37467452, 37775116, 37870536, 38554725, 38834388, 39231341, 39478506.

Trial registry IDs cited by the recommendations: NCT02997358, NCT03016819, NCT03686124, NCT03965234, NCT04482309, NCT04939610, NCT05432791, NCT05551117, NCT05830123, NCT06088290, NCT06242470, NCT06498648, NCT06528769, NCT06551142, NCT06571734, NCT06981637, NCT07156565, NCT07516925, NCT07633756, NCT07686367.